

***NSI* NURSING SOLUTIONS, INC.**

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TOP CEO-COO 2009 CONCERNS

**(Networking Assistance and CEO-COO Job Lead Information,
Recruiter Lists, Resume Writing, Networking Articles,
CEO –COO Compensation, Turnover/Tenure,
and Causes/Effects of Turnover)**

and

HEALTHCARE ECONOMICS REPORT

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CEO TENURE AND TURNOVER RATES

N-689

CEO TENURE

Years

- Average Tenure
- Median Tenure

5.6

3.6

CEO TURNOVER

PERCENTAGE

- 2008
- 2009

16.7%

16.1%

Source: Amalgamation Analysis from: ACHE; AHA; Watson Wyatt; Modern Healthcare; AH; SSI Search Solutions; HR Strategic Solutions, Inc

TOP REASONS FOR CEO TURNOVER

❑ Anticipating Job Change within 5 years*	38.2%
❑ In-Transition	12.9%
❑ Actively Networking	43.6%

**For Free Networking Assistance,
CEO-COO Job Leads, Network Contacts, Salary Data, or
Sample CEO Employment Contracts, call
Marco Colosi at 717-471-7404**

REASONS FOR TURNOVER	%
❑ Career Advancement (Larger or more Complicated Hospital, or Better Environment). Other Voluntary Resignations	16% 11%
❑ In-Voluntary Resignation: (Conflict with Board, Medical Staff, System or Financial Instability) (Source: M. Weirsema, 2002)	71%
❑ Retirement* (25.79% CEOs anticipate retiring over the next 5 years)	2%
Sources: Amalgamation Analysis from: ACHE; William M. Mercer; HR Strategic Solutions; Watson Wyatt	

THE EFFECTS OF CEO TURNOVER

People, Visions, Philosophies, Plans and Management Styles Change, causing Management Turnover and Elevated Market Competitiveness.

IMPACT OF CEO TURNOVER ON:

- | | |
|--|------------|
| ❑ C-Suite (Vice Presidents): | 97% |
| ❑ Chief Medical Officer (CMO): | 77% |
| ❑ Chief Operating Officer (COO): | 52% |
| ❑ Chief Financial Officer (CFO): | 42% |
| ❑ Competitors Raiding of Physicians, Patients and Key Employees: | 39% to 45% |
| ❑ Strategic Plans are Disrupted or Halted: | 30% |
| ❑ New Service Development Stalled/Nixed: | 29% |

Source: HealthLeaders

INSTITUTIONS WITH MULTIPLE CEOs OVER A 5 YEAR PERIOD

Number of CEOs

Percentage

1 CEO:	43%
2 CEOs:	33%
3 CEOs:	17%
4 CEOs:	5%
5 CEOs:	3%

NOTE: THAT 25% of the hospitals reported that their institution had 3 plus CEOs over a 5 year period.

Source: ACHE, The Impact of Hospital CEO Turnover in U.S. Hospitals; HealthLeaders

CEOs RATE THEIR JOB SATISFACTION

- ❑ **Very Unsatisfied with Job:** **1%**
- ❑ **Unsatisfied with Job:** **3%**
- ❑ **Unsatisfied Somewhat with Job:** **16%**
- ❑ **Very Satisfied with Job:** **48.3%**
- ❑ **Satisfied Somewhat with Job:** **31.7%**

Sources: Health Leaders, November, 2007

CEO SUCCESSION PLANNING

	HOSPITAL SYSTEM	
□ No Succession Plan:	73%	51%
□ Have Single Successor Identified:	9%	14%
□ Have Multiple Successors Identified:	18%	35%

Source: ACHE, Succession Planning Practices & Outcomes in U.S. Hospital Systems; HealthLeaders, April, 2008

CEO COMPENSATION 2009 FORECAST

	NET REVENUES			
	\$50M to \$150M	OVER \$200M	OVER \$300M	ALL HOSPITALS
BASE SALARY/TOTAL COMPENSATION N-761				
CEO	\$292.1/\$343.8	\$331.2/\$431.9	\$445.1/\$587.9	\$336.1/\$479
COO	\$185.4/\$214.6	\$273.3/\$331.4	\$346.7/\$431.8	\$288/\$355.9
% INCREASE TOTAL COMPENSATION				
CEO	5.9%	7.4%	9.6%	7.7%
COO	3.3%	6.9%	5.19%	5.1%
Bonus Program Prevalence				
Bonus Type	Hospitals Using	Systems Using	Effectiveness	
Group Incentives	9%	17%	41%	
Lump Sum	31%	13%	53%	
Gain Sharing	42%	16%	55%	
Key Contributor	14%	0	30%	
Profit Sharing	8%	7%	52%	
Source: Amalgamation Analysis from: Towers Perrin; William M. Mercer; Watson Wyatt; Segal & Co; Hay Surveys; HR Strategic Solutions, Inc.; SSI Search Solutions, Inc				

CEO BONUS PROGRAM BASIS

❑ Financial Performance:	72.9%
❑ Quality and Patient Safety:	53.2%
❑ Patient Satisfaction:	50.2%
❑ Operational Efficiency:	44.4%
❑ Employee Satisfaction:	35.9%
❑ Physician Satisfaction:	31.9%

Source: Amalgamation Analysis from: HealthLeaders, November, 2007; Towers Perrin; William M. Mercer; Watson Wyatt; Segal & Co; Hay Surveys; NSI Nursing Solutions, Inc.

CEO CHALLENGES

ISSUES	2008	2009
❑ Market, Economy and Financial challenges	69%	74%
❑ Physician/hospital relations	55%	54%
❑ Personnel Shortages	48%	52%
❑ Care for the uninsured	43%	44%
❑ Quality	33%	37%
❑ Patient safety	32%	34%
❑ Governmental mandates	21%	24%
❑ Patient satisfaction	18%	23%
❑ Capacity	14%	14%

Sources: Amalgamation Analysis from: Modern Healthcare; AHA; ACHE; HR Strategic Solutions; HealthLeaders

COMPETITIVE THREATS AND SOLUTIONS

Competitive Threats that worry CEOs:

- ❑ Loss of Market Share to Competitors: 40.3%
- ❑ Profitable Services Lost to Specialty Facilities: 25.6%
- ❑ Incursion by Large Regional Health Systems: 13.6%
- ❑ Competition from Retail Clinic: 4%

Strategies to address these Competitive Threats:

- ❑ Build Outpatient Radiology Services: 63.8%
- ❑ Build Retail Clinics: 31.2%
- ❑ Implement Concierge Services: 16.7%
- ❑ Spa Services: 12.8%
- ❑ Luxury Units: 5.7%

Sources: HealthLeaders, November 2007; ACHE

HOW CEOs SPEND THEIR TIME

	<10%	10% to 25%	26% to 50%	>50%
Governance Board Relations:	32%	57%	10%	0
Physician Relations:	9%	53%	32%	4%
Community Outreach:	34%	56%	8%	0
Politics/Regulatory:	68%	27%	4%	0
Operations:	10%	43%	38%	8%
Staff/Leadership Development:	33%	55%	10%	1%
Finance/Accounting:	44%	42%	12%	1%
Strategic Planning:	23%	56%	19%	0
Building/Expansion:	57%	33%	7%	1%
Payer/Provider Contracts:	85%	13%	1%	0

Source: HealthLeaders, November, 2007

CEO CHECK LIST: HOW INVOLVED ARE YOU?

Answer Yes No

- **() () Openly engage with medical staff, nursing and other leaders in patient safety planning.**
- **() () Continuously articulate the business case for safety improvement and adequate staffing.**
- **() () Personally participate in a significant incident investigation/root cause analysis.**
- **() () Tell “my story” around incidents/errors that I have been involved with and the improvements that could have prevented them.**
- **() () Involved in direct and ongoing communications on patient safety and improvement areas, at all levels with staff, patients and family members.**
- **() () Refocus hiring and promotional practices to reflect patient quality of care and safety as a priority.**
- **() () Routinely bring patient safety, trend data and specific cases to the board and leadership committees.**
- **() () Assign executives to safety performance improvement, retention, and staffing teams and review outcomes regularly.**

CEO CHECK LIST: HOW INVOLVED ARE YOU?

Answer Yes No

- **() () Routinely probe staff perceptions of risk areas from existing or proposed systems and take immediate actions.**
- **() () When successful safety projects are presented, develop a plan to spread work to the organization.**
- **() () Openly support staff involved in root-cause analysis.**
- **() () Ensure that there is on-going prioritization and achievement of safety improvement objectives.**
- **() () Connect executive performance and compensation to patient safety and quality of care improvements.**
- **() () Ensure that articles on work & family balance, patient and safety regularly appear in my communications.**
- **() () As part of annual budget preparation, ensure resources are funded for priority safety areas.**
- **() () Cultivate media understanding of patient safety and my hospital's efforts to improve safety.**
- **() () Ensure effective systems are in place to assess individual competence and accountability.**
- **() () De-budget contract labor, enhancing morale.**

Source: Strategies for Executives and their Role in Patient Safety, AHA, The Institute of Healthcare Improvement

“Although there is a perceived conflict between quality and cost, improving quality has a positive impact on finances...” HFMA's Healthcare Financial Outlook 2007

RELATING COST AND QUALITY IMPROVEMENT

Improvement Focus	Average Wage	HWPD (Hours Worked per Patient Day)	Patient Days per Admission
Patient Flow	X	X	XX
Adverse Drug Event			XX
Ventilator Event	X	XX	XX
Surgery Scheduling	X	XX	XX
Care Coordination	X		XX
Evidence-Based Care			X
Nursing Processes, Efficiency & Reduced Contract Labor	XX	X	

Note: “X” indicates moderate relationship; “xx” indicates strong relationship

RNs ARE REVENUE GENERATORS

RNs are the “New Rainmakers”

Finance No Longer Considers Nurses as a Cost.

- This is now due to the new:
 - 1) **CMS Inpatient Prospective Payment System**, that disallows reimbursement for hospital acquired conditions or mistakes (“never events”) and,
 - 2) **HCAHPS data on patient satisfaction**, adjusted for case mix, provides consumers a list of measures to compare hospitals.
- RNs are Revenue generators as espoused by NSI since 2001. HFMA and HealthLeaders Media News now agree. RNs are generators of revenues to the tune of:
\$250,000 to \$300,000

*Source: HealthLeaders, <http://www.healthleadersmedia.com/video/content/202667/The-New-Rainmakers.html>

HEALTHCARE ECONOMICS

National Health Expenditure 2009:	\$2,565 Billion
National Health Expenditure 2009 per capita:	\$8,341
ER Visits 2009:	4%+
Hospitals in the Red:	27%
Admissions:	Flat
ALOS:	5.6 days
ER Visits: (119 million in 2006, with 12% of EDs closed)	123.3 million
Number of Hospitals:	5,708
Number of Public Hospitals:	1,119
Number of Children's Hospitals:	263
Number of Investor Hospitals:	889
Number of Not for Profits:	2,919

Sources: AHA; Modern Healthcare; Daily Dose; HealthLeaders; HFMA ; ACHE; Moody's; ACEP Report, 2009

HEALTHCARE COSTS ACCELERATING RAPIDLY

Type of Cost	2009 Healthcare Costs N-510		
	2006	2007	2008
Labor & Benefit Costs per Staff Occupied Bed	\$435,123	\$455,147	\$475,111
Labor & Benefit Expense per FTE	\$74,208	\$81,168	\$83,457
RX Costs per Adjusted Discharge at 50%	\$574	\$631	\$694
Cost of Benefits Increase	7.7% - 9.6%	8.7% - 10.1%	6% - 9%
RN Working in Agencies*	109,354	111,798	101,387 (represents a shrinking pool)

***Represents an estimated \$12,000,000,000 in Industry Cost compared to \$6,000,000,000, with RNs as employees. This represents cost savings of \$76,573/employed RN or an average hospital cost savings of \$1,531,460.**

Sources: Amalgamation Analysis from: William M. Mercer; Hewitt Associates; Watson Wyatt; Hay Group; Segal & Co; Towers Perrin; ACHE; AONE; Staffing Industry Analyst; and HR Strategic Solutions surveys, etc.

HEALTHCARE ECONOMICS

Turnover Rate: (NSI FY annualized 6.3%)*	21%
New Employee First Year Turnover Rate*:	30% to 45%
Cost of RN Turnover:	\$66,778 to \$133,556
Retention Rate: (NSI FY annualized 93.7%)	79%
Vacancy Rate Ranges:	6.1% to 23.2%
Average RN Salary: (\$32.10 per hour)	\$66,778
Travel RN Cost per year/RN (Average):	\$99,840 to \$156,000
Travel RN Cost Saved per 29 RNs employed:	\$1,809,600
RN Cost-per-Hire: Vanderbilt University School of Nursing Dean Welch reports RN Cost per hire at \$40,000 to \$90,000 (The Tennessean, June 7, 07)	\$29,278- \$34,168
RN Recruitment Difficulty Rating:	84.4%
RN Time-to-Fill: (NSI 32 days)	109.9 days

Sources: AHA; Modern Healthcare; Daily Dose; HealthLeaders; HFMA;

EXPENSES PER INPATIENT DAY

Source: Thomson Reuters, June, 2008; HealthLeaders Media, August, 2008

STATE	EXPENSE PER INPATIENT DAY	STATE	EXPENSE PER INPATIENT DAY
AK	\$2,246	CO	\$1,751
WA	\$2,143	MA	\$1,751
OR	\$2,062	RI	\$1,719
CA	\$1,994	DE	\$1,715
DC	\$1,910	CT	\$1,713
MD	\$1,831	NV	\$1,685
UT	\$1,823	OH	\$1,673
NJ	\$1,797	IL	\$1,637
NM	\$1,780	TX	\$1,636
AZ	\$1,769	NH	\$1,127
IN	\$ 1,569	VA	\$1,394
MO	\$1,560	OK	\$1,332
NY	\$1,539	NC	\$1,320

EXPENSES PER INPATIENT DAY

Source: Thomson Reuters, June, 2008; HealthLeaders Media, August, 2008

STATE **EXPENSE PER**
INPATIENT DAY

STATE **EXPENSE PER**
INPATIENT DAY

ME **\$1,528**

MN **\$1,300**

PA **\$1,500**

LA **\$1,293**

FL **\$1,497**

AK **\$1,238**

ID **\$1,484**

TN **\$1,234**

SC **\$1,465**

GA **\$1,202**

MI **\$1,464**

AL **\$1,198**

WI **\$1,458**

KY **\$1,194**

VT **\$1,166**

MS **\$1,021**

WV **\$1,113**

ND **\$898**

NE **\$1,066**

MT **\$814**

KS **\$1,055**

WY **\$805**

IA **\$1,036**

SD **\$733**

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NEED NURSES?

- ***NSI* NURSING SOLUTIONS, INC** is a 9 year old national high volume U.S. nurse recruitment firm; that recruits only within the continental U.S. for American *Experienced Nurses*, as your core employee staff, in an average **time-to-fill of 32 days**.
- ***NSI*** is not a foreign recruiter nor a travel or temporary agency.
- The partnership is "RISK FREE, since you must hire the nurses BEFORE ***NSI*** is paid.
- The relationship is also a "**WIN-WIN**", since not only are the nurses guaranteed for 6 months, ***NSI*** provides "On-Boarding Coaching" for the nurses, for 365 days.